
Deals

The tape stayed busy but shifted downmarket. After a July of multi-billion pipeline takeouts, August's M&A was smaller, more clinical, and more spinout-heavy: buyers paid up for single late-stage rare-disease assets and left the rest behind. Lilly kept its foot down (its thirteenth acquisition of the year), sponsors kept re-rating the plumbing, and McKesson made the biggest bet that had nothing to do with a drug. 2026 has now seen 69 disclosed biotech/biopharma M&A transactions worth roughly \$198.6B combined, running 27 public targets to 42 private.

Eli Lilly Acquires Merida Biosciences for Up to \$2.875B, Buying an Autoantibody-Degrader Platform

On Monday (8/31), Lilly announced a definitive agreement to acquire Merida Biosciences for up to \$2.875 billion in cash, inclusive of an undisclosed upfront and contingent development and regulatory milestones. Expected to close in Q4 2026. It is Lilly's thirteenth acquisition of 2026 and the first therapeutics control transaction in three weeks, ending a stretch where licensing and NewCo formation had been doing all the work.

What Lilly is buying is a mechanism, not a molecule:

- **MER511:** A Phase 1 selective autoantibody degrader in Graves' disease (first patient dosed December 2025), designed to degrade the specific pathogenic autoantibody driving the disease rather than broadly suppressing the immune system.
- **MER769 and a membranous nephropathy program:** MER769 targets IgE-mediated allergic disease, with a primary membranous nephropathy program approaching the clinic, all built on the same antibody-degradation platform.

Paying up to \$2.875B for a single Phase 1 asset plus platform is aggressive on any conventional read, but it is consistent with how Lilly has spent 2026: most of its deals early, most of them mechanism-led. Selective degradation of a pathogenic autoantibody is the autoimmune field's central bet right now, and unlike the CD19 CAR-T programs frozen by safety signals the same week, a degrader carries none of that risk. For Third Rock, it is a clean return on a company that raised \$121M and exited inside eighteen months of leaving stealth.

Structure: Definitive merger agreement; cash upfront plus milestones. Subject to customary conditions and regulatory clearances.

McKesson Acquires Precision Medicine Group for ~\$2.25B, Assembling the Full Oncology Value Chain

On Monday (8/25), McKesson announced a definitive agreement to acquire Blackstone-backed Precision Medicine Group for approximately \$2.25 billion in cash. Expected to close subject to customary conditions and regulatory clearances; the business folds into McKesson's Oncology & Multispecialty segment. This is the piece of the oncology value chain McKesson did not already own.

Precision brings three connected businesses:

- **Precision for Medicine:** A biomarker-driven CRO and specialty-lab network spanning translational science through trial execution.
- **QuartzBio and Precision AQ:** Biomarker data software (QuartzBio) plus market-access, evidence and commercialization consulting (Precision AQ), the front-end biomarker layer and the back-end access layer respectively.

McKesson already controls the sites of care (US Oncology Network), the real-world data (Ontada) and the trial network (SCRI). Precision closes the loop: the same company can now help a sponsor find the biomarker, run the trial, generate the evidence, secure coverage and distribute the drug. Committing \$2.25B of cash into services rather than more drug distribution signals where distributor economics are heading, and it is a second data point after Dassault/ArisGlobal that strategic buyers, not sponsors, are becoming the natural owners of specialized clinical-development assets.

Structure: Definitive agreement; all-cash. Subject to customary conditions and regulatory clearances.

Jazz Pharmaceuticals Acquires Actio Biosciences for Up to \$1.32B, Rebuilding Epilepsy Around Genetics

On Monday (8/10), Jazz announced an agreement to acquire Actio Biosciences for \$820 million in cash at closing plus up to \$500 million in contingent milestones (total up to \$1.32 billion). Expected to close in Q4 2026.

The deal is built around one asset:

- **ABS-1230:** A first-in-class oral small-molecule KCNT1 ion channel inhibitor in the Phase 1b/2a KYRON registrational trial for KCNT1-related developmental and epileptic encephalopathy. It holds FDA Fast Track, Rare Pediatric Disease and Orphan Drug designations and was accepted into the FDA's Rare Disease Evidence Principles (RDEP) program.

Jazz is paying \$820M in cash for a Phase 1b/2a asset in roughly 2,500 US patients, an aggressive price per patient that only works because RDEP offers a credible route to approval on a single small registrational trial. It is a deliberate rebuild of the epilepsy franchise around genetically defined populations rather than another broad-label Epidiolex, and it lands as Jazz needs pipeline ahead of Xywav erosion. The spinout structure is the mechanic to note: Jazz buys precisely the one program it wants, leaves the TRPV4 work (ABS-0871 for Charcot-Marie-Tooth type 2C) with the team that built it, and keeps a minority stake.

Structure: Cash at closing plus contingent milestones; early-pipeline spinout with a Jazz minority stake. Subject to customary conditions.

Tarsus Acquires Alkeus for Up to \$800M, Extending Into Inherited Retinal Disease

On Thursday (8/6), Tarsus Pharmaceuticals agreed to acquire Alkeus Pharmaceuticals for \$450 million at signing (\$270M cash plus \$180M in Tarsus stock) plus up to \$350 million in contingent payments (total up to \$800 million), funded alongside an oversubscribed \$125M PIPE led by Bain Capital Life Sciences and TCGX.

The deal centers on one asset:

- **Gildeuretinol (ALK-001):** A once-daily oral deuterated vitamin A analog in Phase 3 for Stargardt disease, a condition with no approved therapy and roughly 36,000 US patients. The asset previously failed a Phase 3 in geographic atrophy.

Tarsus extends beyond Xdemvy into inherited retinal disease and pays for it without touching the balance sheet: the PIPE comes from Alkeus's own crossover investors, about as close to a self-funding structure as an eye-care bolt-on gets. The catch is competitive position: Belite Bio's tinlarebant has already won its Phase 3 and is in front of the FDA, so Tarsus is buying second position in a race it does not lead, with a pivotal readout not due until 2029.

Structure: Cash-and-stock at signing plus contingent payments, funded alongside a \$125M PIPE. Subject to customary conditions.

BioMarin Acquires Alesta for Up to \$490M, Defending Its Rare-Bone Franchise

On Tuesday (8/18), BioMarin agreed to acquire private Alesta Therapeutics for \$275 million cash upfront plus up to \$215 million in development and regulatory milestones (total up to \$490 million). Expected to close in Q3 2026.

The target is a single program:

- **ALE1:** An orally active small molecule targeting inorganic pyrophosphate (PPi) regulation, the central metabolite in hypophosphatasia (HPP), in an ongoing Phase 1/2a study.

BioMarin is defending a franchise rather than building one. The HPP standard of care is injectable (Alexion's Strensiq, dosed subcutaneously several times a week), so an oral acting directly on PPI changes the burden equation for a chronic, lifelong disease. The structure shows how early this is: \$275M for a Phase 1/2a asset with no efficacy data disclosed, a spun-out remainder and no team transferring. With roughly 9,000 diagnosed US patients, it is a small, defensible, underdiagnosed market where BioMarin already owns the prescriber relationships.

Structure: Cash upfront plus milestones; single-program buy with non-ALE1 assets and employees spun out pre-close. Subject to customary conditions.

Deep Dive: Reading Between The Lines

The Spinout Buy Is Now the Default

Four of this window's five deals used the same structure: buy one asset, spin the rest of the company out to the team that built it, sometimes keeping a minority stake. Jazz did it with Actio's TRPV4 work, BioMarin with Alesta's non-ALE1 assets, and both structured the deal so they paid only for the single program they wanted. This is the maturation of rare-disease M&A into a precision instrument: acquirers no longer buy a company and rationalize it afterward; they carve out the exact asset at signing. It keeps prices per-asset high while capping total outlay, and it leaves founders with a funded second act. Expect more of it.

RDEP Is Quietly Repricing Ultra-Rare Assets

Jazz paid \$820M cash for a Phase 1b/2a asset largely because the FDA's Rare Disease Evidence Principles program offers a path to approval on a single small registrational trial. That is the number to sit with: RDEP is starting to function as a valuation multiplier for genetically defined, ultra-rare programs, because it collapses the usual two-pivotal-trial timeline and de-risks the regulatory path before efficacy is even established. As more programs enter RDEP, the per-patient prices that look aggressive today (Jazz is paying roughly \$330K per addressable patient upfront) become the comp everyone else underwrites against.

McKesson and the Services Land-Grab

The largest deal of the window bought no molecule at all. McKesson's \$2.25B for Precision Medicine Group is the clearest sign yet that the distributors and strategics see margin growth coming from what they know about oncology, not what they ship. Paired with Dassault/ArisGlobal and Repligen/BioLife from July, the through-line is that the CRO, biomarker, and commercialization layers are being consolidated by buyers who already own adjacent pieces, and that specialized clinical-development services are now a strategic asset class rather than a fragmented vendor market.

IPOs

The window opened with a burst and then went quiet. Four biotechs priced inside the first week of August, most of them upsized and most trading above issue, before the IPO tape effectively paused for the rest of the window. As of the latest refresh, 2026 has now seen 22 US-listed biotech IPOs priced for roughly \$6.9 billion in total gross proceeds, with 18 of 22 trading above their offer price.

Braveheart Bio began trading on August 6 after pricing an upsized \$382.5 million IPO at \$18/share, and closed its debut up 66%, the strongest first day of the cohort. The company is walking BHB-1893, an oral cardiac myosin inhibitor licensed from Hengrui, into global Phase 3 for obstructive hypertrophic cardiomyopathy, a China-origin asset with a Phase 3 design already run. It sits at roughly +62% versus its offer price.

Latigo Biotherapeutics and BlossomHill Therapeutics both priced on August 6. Latigo raised \$345.6 million at \$18/share to advance LTG-001, an oral Nav1.8 inhibitor for moderate-to-severe acute pain with a positive 343-patient abdominoplasty Phase 2b published in NEJM; it trades around +25% from issue. BlossomHill raised \$168.3 million at \$16/share for BH-30643, a brain-active, mutant-selective EGFR inhibitor for EGFR- and HER2-mutated NSCLC, and sits around +61%. Vogenx followed on August 11 with an \$81.3 million IPO at \$13/share for mizagliflozin, an oral selective SGLT1 inhibitor for post-bariatric hypoglycemia, and has since run to roughly +150%, one of the year's best aftermarket performers.

The pattern in this cohort is the same one that has defined 2026: the bid is for de-risked assets somebody else already paid to de-risk, whether that is a completed NEJM-published Phase 2b (Latigo) or an in-licensed China-origin candidate with a Phase 3 design already run (Braveheart). Parabilis Medicines remains the largest biotech IPO of 2026 at \$670.0 million, and Veradermics remains the clear aftermarket winner at north of +500% from its issue price.

Licensing Deals

Genentech Licenses Hanmi's Muscle-Preservation Peptide for Up to ~\$2.5B

On Monday (8/24), Genentech (Roche) took exclusive worldwide rights excluding South Korea to HM17321, a CRFR2-selective urocortin-2 analogue designed to increase lean muscle mass while reducing fat mass, from Hanmi Pharmaceutical.

- **Mechanism and stage:** A non-incretin peptide in Phase 1. Unlike myostatin blockers that spare muscle, CRFR2 agonism aims to add it, which if it holds in humans gives a combination story rather than a defensive add-on to incretin therapy.
- **Deal terms:** \$190 million upfront plus up to roughly \$2.1 billion in milestones (a package of up to about \$2.3 billion), plus tiered royalties. Hanmi completes the ongoing Phase 1 and keeps Korean rights; Genentech leads Phase 2 onward. It is Hanmi's largest out-licensing deal to date.
- **Why it matters:** The muscle-preservation trade has moved from thesis to auction. Every incretin franchise now carries the same overhang (roughly a quarter of the weight lost is lean mass), and the field is buying the answer rather than building it. Roche needed this: CT-388 and petrelintide put it in the incretin race, but without a muscle asset it was fighting the next label war a step behind.

SK Biopharmaceuticals Licenses Biohaven's Kv7 Epilepsy Asset for Up to \$795M

On Tuesday (8/26), SK Biopharmaceuticals took a worldwide license to opakalim (BHV-7000), Biohaven's Kv7.2/Kv7.3 channel activator, plus Biohaven's wider Kv7 openers and discovery platform.

- **Mechanism and stage:** A selective Kv7 potassium-channel activator without GABA-A activity, in Phase 2/3 for focal-onset epilepsy, with the RISE3 focal-epilepsy topline expected H2 2026 and mood and pain indications behind it.
- **Deal terms:** \$400 million in non-refundable near-term cash (\$350M at closing, \$50M after one year) plus up to \$395 million more, including SK assuming up to \$245 million of Biohaven's existing Knopp obligations, for a headline of up to \$795 million; plus tiered royalties. SK targets a 2029 US launch.
- **Why it matters:** SK is the only company in this mechanism with a commercial epilepsy franchise already built (Xcopri gave it the sales force and the neurologist call list), so it is buying the asset that most cheaply fills the second slot in a channel it already sells into. Paying \$400M of near-term cash ahead of Biohaven's own RISE3 readout is an unusually confident pre-data mark, and it makes the Kv7 category a two-horse race against Xenon's azetukalner with a commercial operator on one side.

Sobi Licenses Innate's Lacutamab for Up to \$580M in Rare Blood Cancer

On Monday (8/10), Sobi took a global license to lacutamab, Innate Pharma's first-in-class anti-KIR3DL2 antibody for cutaneous T-cell lymphoma (Sezary syndrome and mycosis fungoides).

- **Deal terms:** \$75 million cash at closing, up to \$40 million in near-term Sezary milestones, and up to \$465 million tied to Sobi's option to take full development rights plus future milestones (\$580 million in aggregate), with tiered double-digit royalties. Innate runs the TELLOMAK-3 confirmatory Phase 3; Sobi commercializes on accelerated approval.
- **Why it matters:** Innate spent years looking for a partner willing to fund lacutamab through registration and found one that pays for the shot without taking the trial away. The \$75M at closing is material relative to Innate's market cap and effectively resets its runway. For Sobi, KIR3DL2 is a clean rare-disease fit: Sezary syndrome is a

few thousand patients globally with no targeted approved option, exactly the label profile Sobi commercializes well.

Clinical Trials and Data

Arrowhead's Plozasiran Delivers Up to 81% Triglyceride Reduction in Phase 3 SHASTA-3 and -4

At the European Society of Cardiology (ESC) Congress in Munich on August 30, Arrowhead Pharmaceuticals presented detailed Phase 3 data from the global SHASTA-3 and SHASTA-4 studies of plozasiran, an RNAi therapeutic, in patients with severe hypertriglyceridemia (sHTG), as a HOT LINE late-breaker.

- **Results:** Both trials met the primary endpoint of triglyceride reduction versus placebo and all prespecified secondary endpoints, with deep, durable median triglyceride reductions of 79% (SHASTA-3) and 81% (SHASTA-4) from baseline. A statistically significant 78% reduction in acute pancreatitis events versus placebo was observed across the sHTG population, with a 100% event reduction in the highest-risk patients (triglycerides above 880 mg/dL with a prior pancreatitis history).
- **Why it matters:** The acute-pancreatitis signal is the story, not just the lipid numbers: sHTG is dangerous mainly because of pancreatitis risk, and a near-total reduction in the highest-risk group is the kind of hard-outcome data that supports a differentiated label. It strengthens the RNAi case in cardiometabolic disease beyond the crowded LDL and Lp(a) fields.

Summit and Akeso Report Ivonescimab's First Overall-Survival Win Beyond Lung Cancer

In late August, Summit Therapeutics and China-based partner Akeso announced positive Phase 3 data for ivonescimab, a PD-1/VEGF bispecific antibody, in biliary tract cancer, the first readout for the drug beyond non-small cell lung cancer to show a significant overall survival benefit.

- **Why it matters:** Ivonescimab's whole investment thesis has rested on whether its NSCLC results generalize to other tumors. An overall-survival win (the hardest endpoint) in a second, difficult indication is exactly the read-through the bull case needed, and it lands as the PD-1/VEGF bispecific class becomes one of the most closely watched races in oncology. It also continues the pattern of China-origin oncology assets producing globally relevant Phase 3 data.

Macro: A Dense August Approval Calendar, Led by First-in-Class Firsts

Takeda's Orzeyful (oveporexton) Becomes the First Orexin Agonist Approved for Narcolepsy Type 1

On August 5, FDA approved Takeda's oveporexton (Orzeyful), an oral orexin receptor 2 (OX2R)-selective agonist, for narcolepsy type 1 in adults. It is the first medicine to treat NT1 as a unified disorder by directly targeting the loss of orexin signaling that causes the disease, rather than managing individual symptoms. Approval was based on two 12-week placebo-controlled studies (FirstLight and RadiantLight, 273 adults combined), where the 2 mg dose met its primary endpoint on the Maintenance of Wakefulness Test ($p < 0.001$ in both). The approval validates a mechanism the sector has chased for years and lands the same week Lilly closed its own sleep-medicine build-out, underscoring how competitive the orexin space has become.

Moderna's mFlusiva Clears as an mRNA Flu Vaccine, and Replimune's Tudriqev Wins on Resubmission

Also on August 5, FDA approved Moderna's mFlusiva (mRNA-1010), an mRNA influenza vaccine, extending the mRNA platform beyond COVID into seasonal flu after a unanimous advisory-panel vote in June. The next day, August 6, FDA granted accelerated approval to Replimune's Tudriqev (vusolimogene oderparepvec), an oncolytic viral therapy, in combination with nivolumab for advanced cutaneous melanoma, a notable turnaround given the program's earlier regulatory setback. Two platform-defining approvals (an mRNA vaccine and an oncolytic virus) inside 48 hours is a useful signal that novel-modality review is moving on schedule.

An Oncology-Heavy Second Half of the Month

The back half of August was dense with cancer approvals: BMS's Zenbuxus (iberdomide) for multiple myeloma and a tau PET imaging agent (Tauklarity) on August 13; Jazz/Zymeworks' Ziihera (zanidatamab) for HER2-positive gastric cancer and Revolution Medicines' Rasonque (daraxonrasib) for metastatic pancreatic cancer on August 25-26; and the first-in-class approvals of rusfertide (Mimrylo) for polycythemia vera on August 28 and brepocitinib (Lisraya), the first oral drug for dermatomyositis, on August 27. The concentration of first-in-class and first-in-disease approvals in a single month is the throughline, and all of it landed while the agency continued to operate under acting Commissioner Kyle Diamantas with several senior review-center seats still filled on an acting basis.

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